

SUPERVISED VOCATIONAL QUALIFICATION DIPLOMA - EXAMINATION

Session: Revision Mock Set 10	Duration: 3 hours
Specialty: Psychotherapy	Coefficient: 04
Eliminatory mark: < 8/20	Training reference: VTI-AEDA examination style

COMPREHENSIVE VOCATIONAL PAPER - SOLUTION / MARKING GUIDE PRACTICE SET 10

Revision material based on the supplied VTI-AEDA 2025 examination format and the supplied psychotherapy course recordings. Not an official examination paper.

How to use this marking guide

For revision, first attempt the paper without notes. Then compare not only the answer letter but the clinical cue that makes the answer correct. For legal/ethical items, the guide follows the concepts emphasized by the supplied course/examination materials; in real practice, applicable Cameroon law, institutional policy and the current professional code must also be checked.

PART ONE: PSYCHOPATHOLOGY - ANSWERS

1. A therapist notes that Patricia has euphoria, increased energy and confidence, decreased need for sleep, agitation and autonomic activation. Which condition should be considered first from the options below?

- A. Schizoid personality disorder
- B. Stimulant intoxication**
- C. Generalized anxiety disorder (GAD)
- D. Social anxiety disorder

Answer: B - Stimulant intoxication. Stimulants activate rather than sedate; cocaine and amphetamines commonly produce energy, alertness and agitation.

Memory aid: *STIMULANT = STIMulates.*

2. A therapist notes that Samuel has pervasive instability in relationships, self-image and affect, impulsivity, abandonment fears and recurrent self-harm or suicidal behavior. Which condition should be considered first from the options below?

- A. Borderline personality disorder**
- B. Opioid intoxication
- C. Anorexia nervosa, restricting type
- D. Social anxiety disorder

Answer: A - Borderline personality disorder. Borderline personality disorder is marked by instability and intense reactivity across relationships, identity, mood and behavior.

Memory aid: *BORDER: Bonds unstable, Overreactive affect, Risky acts, Desertion fear, Empty/identity shifts, Recurrent self-harm.*

3. During assessment, Tina reports perfectionism, orderliness, rigidity and control that interfere with flexibility and task completion. Which of the following is the BEST match?

- A. Manic episode
- B. Opioid intoxication
- C. Schizoid personality disorder
- D. Obsessive-compulsive personality disorder (OCPD)**

Answer: D - Obsessive-compulsive personality disorder (OCPD). OCPD is a personality pattern of perfectionism and control; unlike OCD, true obsessions and compulsions are not required.

Memory aid: *OCPD = Order, Control, Perfectionism, Duty.*

4. A therapist notes that Wendy has detachment from social relationships and restricted emotional expression with little desire for close relationships. Which condition should be considered first from the options below?

- A. Schizoid personality disorder**

- B. Anorexia nervosa, binge-eating/purging type
- C. Borderline personality disorder
- D. Bipolar disorder

Answer: A - Schizoid personality disorder. The key pattern is social detachment and limited affect, rather than fear of rejection as in avoidant personality.

Memory aid: SCHIZOID = Solitary by preference.

5. During assessment, Brenda reports excessive need to be cared for, difficulty making decisions independently, submissiveness and fear of separation. Which of the following is the BEST match?

- A. Dependent personality disorder**
- B. Opioid intoxication
- C. Substance use disorder
- D. Schizophrenia

Answer: A - Dependent personality disorder. Dependent personality is characterized by reliance on others for decisions, reassurance and support.

Memory aid: DEPENDENT = Decisions Entrusted to People; separation fear.

6. Patricia, a 22-year-old teacher, presents with persistent social-communication difficulties together with restricted or repetitive patterns of behavior or interests beginning in development. Which diagnosis or clinical concept best fits this presentation?

- A. Substance use disorder
- B. Opioid intoxication
- C. Autism spectrum disorder**
- D. Bipolar disorder

Answer: C - Autism spectrum disorder. Autism requires both social-communication differences and restricted/repetitive behavior patterns.

Memory aid: 2R domains: Relating/communication + Repetitive/restricted behavior.

7. Samuel, a 47-year-old university student, presents with recurrent unexpected panic attacks followed by persistent worry about more attacks or maladaptive avoidance. Which diagnosis or clinical concept best fits this presentation?

- A. Borderline personality disorder
- B. Generalized anxiety disorder (GAD)
- C. Obsessive-compulsive personality disorder (OCPD)
- D. Panic disorder**

Answer: D - Panic disorder. Panic disorder is more than a single panic attack: recurrent unexpected attacks are followed by ongoing concern or behavior change.

Memory aid: PANIC: Panic Attacks + New worry/avoidance Is Continuing.

8. A therapist notes that Amina has excessive, difficult-to-control worry across several life areas for about six months, with tension, restlessness or poor sleep. Which condition should be considered first from the options below?

- A. Obsessive-compulsive disorder (OCD)
- B. Generalized anxiety disorder (GAD)**
- C. Post-traumatic stress disorder (PTSD)
- D. Anorexia nervosa, restricting type

Answer: B - Generalized anxiety disorder (GAD). GAD is characterized by persistent generalized worry across multiple domains rather than fear tied to one object or situation.

Memory aid: GAD = General Anxiety across Domains.

9. During assessment, Grace reports marked fear of a specific object or situation that triggers immediate anxiety and avoidance out of proportion to the actual danger. Which of the following is the BEST match?

- A. Agoraphobia
- B. Specific phobia**

- C. Stimulant intoxication
- D. Binge-eating disorder

Answer: B - Specific phobia. A specific phobia is circumscribed: the fear is linked to a particular object or situation and typically produces avoidance.

Memory aid: *SPECIFIC = one SPECIFIC trigger.*

10. A therapist notes that Kemi has marked fear of social or performance situations because of possible scrutiny, embarrassment or negative evaluation. Which condition should be considered first from the options below?

- A. Dependent personality disorder
- B. Specific phobia
- C. Schizophrenia
- D. Social anxiety disorder**

Answer: D - Social anxiety disorder. Social anxiety centers on being observed, judged or embarrassed in social/performance settings.

Memory aid: *SOCIAL = Scrutiny Of Client In Activities with Lots of people.*

SECTION B: TRUE/FALSE - EXPLANATIONS

1. TRUE. Reduced need for sleep, grandiosity and pressured speech are compatible with mania.

Those are classic manic activation features.

2. FALSE. Bulimia nervosa necessarily requires markedly low body weight.

Markedly low weight is central to anorexia; bulimia can occur at many body weights.

3. TRUE. Binge-eating disorder is defined by recurrent binge eating without regular compensatory behaviors.

Regular purging or other compensation would point more toward bulimia if other criteria are met.

4. FALSE. OCPD and OCD are identical diagnoses.

OCPD is a personality pattern of perfectionism/control; OCD centers on obsessions and compulsions.

5. TRUE. Borderline personality disorder may include abandonment fears and recurrent self-injury.

Both are characteristic features of borderline personality disorder.

SECTION C: MODEL SHORT ANSWERS

1. Differentiate a panic attack from panic disorder. **(2mks)**

- A panic attack is an acute episode of intense fear/discomfort with physical/cognitive symptoms.
- Panic disorder requires recurrent unexpected attacks plus persistent concern and/or maladaptive behavior change.

Memory aid: *Attack = episode; disorder = recurring pattern + aftermath.*

2. Differentiate obsession from compulsion and give one example of each. **(2mks)**

- Obsession: recurrent intrusive unwanted thought/urge/image, e.g. fear of contamination.
- Compulsion: repetitive behavior/mental act to reduce distress, e.g. repeated hand washing.

Memory aid: *O = thought; C = conduct.*

3. State the four D criteria commonly used when considering abnormality. **(2mks)**

- Deviance
- Distress
- Dysfunction
- Danger

Memory aid: *4D: Deviance, Distress, Dysfunction, Danger.*

4. Define substance use disorder in practical clinical terms. **(2mks)**

- Problematic pattern of substance use causing significant impairment or distress.
- Look for loss of control, social/role impairment, risky use and/or tolerance/withdrawal.

Memory aid: *4C: Control loss, Consequences, Craving, Continued use.*

5. Give two reasons early intervention in substance use disorder is important. (2mks)

- Reduces medical, social, occupational and legal harm and risk of escalation.
- Improves opportunity for engagement, relapse prevention, family support and recovery before consequences accumulate.

Memory aid: EARLY = less Entrenchment, Address Risk, Link help, Yield recovery.

PART TWO: THEORIES/TECHNIQUES - ANSWERS

1. Which psychotherapy concept is most accurately described by the following: a behavior increases because it removes or prevents an aversive stimulus?

- A. Gestalt therapy
- B. Flooding
- C. Negative reinforcement**
- D. In vivo exposure

Answer: C - Negative reinforcement. Negative reinforcement strengthens behavior; negative means something unpleasant is removed, not that behavior is punished.

Memory aid: Reinforcement always raises behavior; negative removes something.

2. In a session with Rose, the counselor emphasizes a previously reinforced behavior decreases when the reinforcement maintaining it is consistently withheld. This BEST illustrates:

- A. Exposure therapy
- B. Extinction**
- C. Gestalt dream work
- D. Systematic desensitization

Answer: B - Extinction. Extinction occurs when reinforcement is no longer provided for a behavior.

Memory aid: Extinction = extinguish the payoff.

3. In a session with Quentin, the counselor emphasizes learning behavior by observing what happens to another person or by imitating a model. This BEST illustrates:

- A. Social modeling**
- B. Person-centered therapy
- C. Retroflexion
- D. Rational Emotive Behavior Therapy (REBT)

Answer: A - Social modeling. Modeling is observational learning: the client learns from another person's behavior and its consequences.

Memory aid: MODEL = learn by watching a model.

4. In a session with Uma, the counselor emphasizes planned contact with feared stimuli, memories or situations while reducing avoidance so fear can be relearned. This BEST illustrates:

- A. Psychoanalytic therapy
- B. Cognitive behavioral therapy (CBT)
- C. Transference
- D. Exposure therapy**

Answer: D - Exposure therapy. Exposure reduces avoidance and allows corrective learning; it may be in vivo, imaginal or other formats.

Memory aid: Approach, do not avoid.

5. Which psychotherapy concept is most accurately described by the following: direct, real-life exposure to a feared object or situation?

- A. Psychoanalytic therapy
- B. Systematic desensitization
- C. In vivo exposure**
- D. Social modeling

Answer: C - In vivo exposure. In vivo means exposure in the actual situation rather than only in imagination.

Memory aid: VIVO = in real life.

6. In a session with Uma, the counselor emphasizes gradual exposure to a fear hierarchy combined with relaxation or another incompatible response. This BEST illustrates:

- A. Psychoanalytic therapy
- B. Systematic desensitization**
- C. Flooding
- D. Social modeling

Answer: B - Systematic desensitization. Systematic desensitization classically combines relaxation training with stepwise movement through a fear hierarchy.

Memory aid: RHE: Relax, Hierarchy, Expose gradually.

7. A therapist working with Amina uses an intervention characterized by rapid and prolonged exposure to a high-intensity feared stimulus or situation without gradual hierarchy progression. Which approach or technique is being used?

- A. Gestalt dream work
- B. Flooding**
- C. Retroflexion
- D. Adlerian therapy

Answer: B - Flooding. Flooding differs from graded exposure because the client confronts a high level of fear more rapidly.

Memory aid: Flooding = jump into the deep end.

8. A therapist working with Zachary uses an intervention characterized by systematically tensing and releasing muscle groups to learn physical relaxation and reduce arousal. Which approach or technique is being used?

- A. Empty-chair technique
- B. Interpretation
- C. Progressive muscle relaxation**
- D. Transference

Answer: C - Progressive muscle relaxation. PMR builds awareness of tension and relaxation by working through muscle groups.

Memory aid: Tense - notice - release - breathe.

9. Which psychotherapy concept is most accurately described by the following: a skills-based therapy especially associated with emotion dysregulation and borderline personality disorder, using mindfulness, distress tolerance, emotion regulation and interpersonal effectiveness?

- A. Progressive muscle relaxation
- B. Resistance
- C. Rational Emotive Behavior Therapy (REBT)
- D. Dialectical Behavior Therapy (DBT)**

Answer: D - Dialectical Behavior Therapy (DBT). DBT combines acceptance and change strategies and teaches concrete skills for severe emotion dysregulation.

Memory aid: M-D-E-I: Mindfulness, Distress tolerance, Emotion regulation, Interpersonal effectiveness.

10. A therapist working with Victor uses an intervention characterized by behavioral teaching and rehearsal of interpersonal skills such as greetings, eye contact, listening and conversation. Which approach or technique is being used?

- A. Gestalt dream work
- B. Operant conditioning
- C. Interpretation
- D. Social skills training**

Answer: D - Social skills training. Social skills training uses instruction, modeling, rehearsal and feedback to build interpersonal behavior.

Memory aid: IMRF: Instruction, Modeling, Rehearsal, Feedback.

11. In a session with Fidel, the counselor emphasizes the client says whatever thoughts, feelings, images or memories come to mind without censoring them. This BEST illustrates:

- A. Systematic desensitization
- B. Extinction**

C. Interpretation

D. Free association

Answer: D - Free association. Free association is a classic psychoanalytic technique intended to reduce censorship and reveal unconscious material.

Memory aid: *Psychoanalytic FIRT: Free association, Interpretation, Resistance, Transference.*

12. A therapist working with Esther uses an intervention characterized by the therapist explains or offers meaning about patterns, defenses, dreams, resistance or behavior. Which approach or technique is being used?

A. Person-centered therapy

B. Interpretation

C. Existential therapy

D. Extinction

Answer: B - Interpretation. Interpretation is the analyst's attempt to make latent or previously unconscious meaning more understandable to the client.

Memory aid: *Interpretation = therapist offers possible meaning.*

13. In a session with Grace, the counselor emphasizes the client unconsciously avoids material or processes that threaten to bring unacceptable conflict into awareness. This BEST illustrates:

A. Social skills training

B. Adlerian therapy

C. Resistance

D. Transference

Answer: C - Resistance. Resistance is the defensive blocking of threatening material or therapeutic work.

Memory aid: *Resistance = psychological brakes.*

14. Which psychotherapy concept is most accurately described by the following: the client redirects feelings, expectations or relationship patterns from important past relationships onto the therapist?

A. Resistance

B. Exposure therapy

C. Person-centered therapy

D. Transference

Answer: D - Transference. Transference originates primarily from the client's earlier relationships and becomes enacted in the therapeutic relationship.

Memory aid: *TRANSference = client TRANSfers old relationship feelings to therapist.*

15. Which psychotherapy concept is most accurately described by the following: the therapist's emotional reactions to a client, especially reactions influenced by the therapist's own history or unresolved issues?

A. Person-centered therapy

B. Gestalt dream work

C. Adlerian therapy

D. Countertransference

Answer: D - Countertransference. Countertransference concerns the therapist's responses to the client and must be monitored through self-awareness and supervision.

Memory aid: *COUNTER = counselor side of the relational reaction.*

SECTION B: TRUE/FALSE - EXPLANATIONS

1. **FALSE.** Negative reinforcement weakens behavior.

All reinforcement strengthens behavior; negative reinforcement does so by removing an aversive condition.

2. **TRUE.** Extinction involves withholding the reinforcement that has maintained a behavior.

When reinforcement stops, the behavior may gradually decrease.

3. **TRUE.** Systematic desensitization classically combines relaxation with graded exposure.

The client learns relaxation and moves through a fear hierarchy.

4. **FALSE.** Flooding is always more gradual than systematic desensitization.

Flooding is deliberately high-intensity and comparatively rapid.

5. **TRUE.** Gestalt therapy emphasizes present-moment awareness.

Here-and-now awareness is central to Gestalt work.

SECTION C: MODEL SHORT ANSWERS

1. What is cognitive restructuring? (2mks)

- Identify unhelpful/biased thoughts, examine evidence, and generate more balanced alternatives, then test them behaviorally when appropriate.

Memory aid: Catch - Check - Change.

2. State two principles emphasized in Adlerian therapy. (2mks)

- Any two: behavior is purposeful/goal-directed; social relatedness/social interest; conscious choice; responsibility; lifestyle; encouragement.

Memory aid: GSCR.

3. What does extinction mean in behavior therapy? (1mks)

- A reinforced behavior decreases when the reinforcement maintaining it is consistently withheld.

Memory aid: Extinguish the payoff.

PART THREE: ETHICAL/LEGAL - ANSWERS

1. A counselor asks which principle or professional concept is defined by taking reasonable protective action when a client presents a serious, credible threat toward an identifiable person, as required by applicable law and ethics. The BEST answer is:

- A. Professional competence
- B. Mandated reporting
- C. Beneficence
- D. Duty to warn/protect**

Answer: D - Duty to warn/protect. The course tests duty to warn/protect as an exception to confidentiality when serious danger to others is present. Actual legal requirements vary by jurisdiction.

Memory aid: *TIP: Threat serious, Identifiable target, Protect reasonably.*

2. In professional practice, a situation calls for reporting suspected abuse or other legally reportable situations to designated authorities when law or professional duty requires it. Which ethical/legal concept is MOST directly involved?

- A. Mandated reporting**
- B. Nonmaleficence
- C. Technology ethics
- D. Duty to warn/protect

Answer: A - Mandated reporting. Mandated reporting is a recognized limit to confidentiality; therapists should explain this limit during informed consent.

Memory aid: *Report when law makes reporting mandatory.*

3. During supervision, Hassan describes professional negligence in which a duty is breached and the breach causes harm. Which concept should guide the discussion?

- A. Confidentiality
- B. Malpractice**
- C. Autonomy
- D. Duty to warn/protect

Answer: B - Malpractice. Malpractice requires more than an unfavorable outcome: there must be a professional duty, breach, harm and causal link.

Memory aid: *DBHC: Duty, Breach, Harm, Causation.*

4. In professional practice, a situation calls for practicing only within areas supported by appropriate education, training, supervised experience and current skill. Which ethical/legal concept is MOST directly involved?

- A. Subpoena response
- B. Record keeping
- C. Professional competence**
- D. Duty to warn/protect

Answer: C - Professional competence. Using techniques without adequate training can expose clients to harm and the professional to ethical or legal liability.

Memory aid: *Know your scope; seek supervision or refer.*

5. In professional practice, a situation calls for a situation in which the therapist has another significant role or relationship with the client in addition to therapy. Which ethical/legal concept is MOST directly involved?

- A. Dual or multiple relationship**
- B. Technology ethics
- C. Sexual relationship with a current client
- D. Justice

Answer: A - Dual or multiple relationship. Multiple relationships are not automatically harmful in every context, but should be avoided or carefully managed when they risk exploitation, impaired objectivity or harm.

Memory aid: *Two roles = review power, objectivity, exploitation and harm.*

6. In professional practice, a situation calls for a departure from customary therapeutic boundaries that may be clinically appropriate and non-exploitative in context. Which ethical/legal concept is MOST directly involved?

- A. Professional competence
- B. Duty to warn/protect
- C. Mandated reporting

D. Boundary crossing

Answer: D - Boundary crossing. A boundary crossing is not automatically unethical; context, culture, intent and effect matter.

Memory aid: Crossing may be thoughtful; violation causes or risks serious harm/exploitation.

7. A counselor asks which principle or professional concept is defined by a serious harmful or exploitative breach of therapeutic boundaries. The BEST answer is:

- A. Gift decision-making
- B. Sexual relationship with a current client
- C. Professional competence

D. Boundary violation

Answer: D - Boundary violation. Boundary violations exploit the therapeutic relationship or cause significant harm and are unethical.

Memory aid: Violation = harm/exploitation.

8. In professional practice, a situation calls for sexual or romantic involvement with a current therapy client. Which ethical/legal concept is MOST directly involved?

- A. Confidentiality
- B. Sexual relationship with a current client**
- C. Termination and referral
- D. Record keeping

Answer: B - Sexual relationship with a current client. Sexual relationships with current clients are prohibited because of power imbalance and exploitation risk. Terminating therapy to start a relationship does not erase those concerns.

Memory aid: Current client + sex = no.

9. In professional practice, a situation calls for considering clinical meaning, value, timing, culture, power and exploitation risk before accepting or declining a client gift. Which ethical/legal concept is MOST directly involved?

- A. Sexual relationship with a current client
- B. Autonomy
- C. Gift decision-making**
- D. Dual or multiple relationship

Answer: C - Gift decision-making. Gifts require contextual judgment. A culturally meaningful modest gift may be handled differently from an expensive or coercive gift.

Memory aid: VALUE: Value, Understanding/meaning, Local culture, Unequal power, Effect on therapy.

10. A counselor asks which principle or professional concept is defined by accepting goods or services instead of money when ethically permissible, not exploitative and not clinically contraindicated. The BEST answer is:

- A. Fidelity
- B. Technology ethics
- C. Veracity

D. Bartering

Answer: D - Bartering. Bartering may be acceptable under some codes when not exploitative and when arrangements are clear and clinically appropriate.

Memory aid: Barter only if Boundaries and Benefits are safe.

11. A counselor asks which principle or professional concept is defined by maintaining accurate, timely, secure and clinically appropriate client records. The BEST answer is:

- A. Subpoena response

B. Record keeping

- C. Confidentiality
- D. Ethical response to colleague misconduct

Answer: B - Record keeping. Records support continuity of care, accountability and legal/ethical duties; access must be controlled and retention rules followed.

Memory aid: ACTS: Accurate, Current, Timely, Secure.

12. A counselor asks which principle or professional concept is defined by ending therapy responsibly with preparation, continuity planning and referral when needed rather than abandoning the client. The BEST answer is:

- A. Duty to warn/protect
- B. Autonomy
- C. Confidentiality

D. Termination and referral

Answer: D - Termination and referral. Ethical termination includes adequate notice, discussion, referrals or continuity plans when appropriate.

Memory aid: PLAN: Prepare, Link/referral, Address feelings, Next steps.

13. During supervision, Daniel describes using ethical codes, consultation/supervision and appropriate reporting procedures when credible serious misconduct is observed. Which concept should guide the discussion?

- A. Malpractice
- B. Technology ethics
- C. Ethical response to colleague misconduct**
- D. Nonmaleficence

Answer: C - Ethical response to colleague misconduct. Serious ethical violations should not simply be ignored; follow code, organizational and legal procedures while protecting clients and due process.

Memory aid: C-A-R-E: Consult code, Assess facts, Report appropriately, Ensure client safety.

14. In professional practice, a situation calls for protecting privacy, competence, informed consent, identity, emergency planning and data security when using digital or remote services. Which ethical/legal concept is MOST directly involved?

- A. Sexual relationship with a current client
- B. Technology ethics**
- C. Record keeping
- D. Fidelity

Answer: B - Technology ethics. Technology changes the medium, not the professional duty. Security, consent and contingency planning remain essential.

Memory aid: SPACE: Security, Privacy, Access/identity, Consent, Emergency plan.

15. A counselor asks which principle or professional concept is defined by not automatically disclosing the entire file on receipt of a subpoena; consult applicable law, the client/attorney and professional guidance, and disclose only as authorized or legally required. The BEST answer is:

- A. Gift decision-making
- B. Duty to warn/protect
- C. Subpoena response**
- D. Technology ethics

Answer: C - Subpoena response. A subpoena and a court order are not always identical. The safest course-based principle is not to reflexively release all records; seek proper legal/ethical guidance and protect confidentiality to the extent permitted.

Memory aid: STOP: See legal guidance, Tell/consult client, Only required information, Protect confidentiality.

SECTION B: MODEL SHORT ANSWERS

1. What should a therapist do when credible serious unethical behavior by a colleague is suspected? (2mks)

- Clarify facts, consult the code/supervisor as appropriate, protect the client and follow formal reporting procedures when required.

Memory aid: CARE: Consult, Assess, Report, Ensure safety.

2. Differentiate privacy from confidentiality. (1mks)

- Privacy is the client's right to control access to self and personal information.
- Confidentiality is the professional duty to protect information obtained in the professional relationship.

Memory aid: Privacy = person's right; confidentiality = professional's duty.

3. Differentiate confidentiality from privileged communication. (1mks)

- Confidentiality is an ethical/professional duty.
- Privilege is a legal evidentiary protection against compelled disclosure in certain proceedings and depends on jurisdiction.

Memory aid: Privilege = courtroom shield.

4. Define malpractice. (1mks)

- Professional negligence in which a duty is breached and the breach causes harm; commonly considered through duty, breach, causation and damages/harm.

Memory aid: DBHC.

SECTION C: MODEL SHORT ESSAYS

1. Give two elements of informed consent. (2mks)

- Examples: nature/purpose of services, risks, benefits, alternatives, fees, therapist qualifications, limits of confidentiality, voluntary choice, right to ask questions.

Memory aid: BRALC.

2. When is termination ethically appropriate? (2mks)

- When goals are met, treatment is no longer beneficial/needed, the therapist cannot provide competent service, or other legitimate reasons exist - with adequate planning to avoid abandonment.

Memory aid: PLAN.

3. State one key ethical requirement for clinical records. (1mks)

- Records should be accurate, timely, relevant and securely stored with access protected.

Memory aid: ACTS.

PART FOUR: CULTURE / GROUP / COUNSELING PROCESS / TREATMENT PLAN - ANSWERS

1. Which counseling/group/cultural concept is BEST described by: reviewing progress, consolidating coping skills, planning relapse prevention or referral and ending the professional relationship responsibly?

- A. Open-ended questioning
- B. Termination and follow-up**
- C. Rapport building
- D. Cultural competence

Answer: B - Termination and follow-up. Termination is a therapeutic phase, not simply stopping appointments.

Memory aid: *RECAP: Review, Evaluate, Consolidate, Aftercare, Part.*

2. A trainee observes attending closely to verbal and nonverbal communication and showing the client that the message has been heard and understood. This is an example of:

- A. Cultural imposition
- B. Storming stage
- C. Open-ended questioning
- D. Active listening**

Answer: D - Active listening. Active listening includes presence, minimal encouragers, reflection, clarification and appropriate nonverbal attending.

Memory aid: *SOLER: Squarely face, Open posture, Lean, Eye contact, Relaxed.*

3. A trainee observes identifying and feeding back the emotional meaning expressed by the client. This is an example of:

- A. Reflection of feeling**
- B. Treatment plan
- C. Group role
- D. Open-ended questioning

Answer: A - Reflection of feeling. Reflection of feeling helps clients recognize and explore emotions rather than only facts.

Memory aid: *Name the feeling, not just the facts.*

4. Which counseling/group/cultural concept is BEST described by: restating the essence of the client's content in fresh words without changing its meaning?

- A. Cultural competence
- B. Group confidentiality
- C. Paraphrasing**
- D. Open-ended questioning

Answer: C - Paraphrasing. Paraphrasing checks understanding and encourages elaboration.

Memory aid: *Paraphrase = same meaning, new words.*

5. A trainee observes questions that invite elaboration rather than a yes/no answer. This is an example of:

- A. Open-ended questioning**
- B. Psychoeducational group
- C. Case formulation
- D. Self-disclosure

Answer: A - Open-ended questioning. Open questions support exploration, while excessive questioning can turn therapy into an interrogation.

Memory aid: *WHAT/HOW usually open more than DID/IS.*

6. In counseling practice, Irene encounters a situation involving respectfully pointing out discrepancies between a client's words, behaviors, values or nonverbal cues. Which concept BEST applies?

- A. Rapport building
- B. Active listening**

C. Paraphrasing

D. Therapeutic confrontation

Answer: D - Therapeutic confrontation. Confrontation is not aggression; it is a careful invitation to examine inconsistency.

Memory aid: *Confront discrepancy, not the person.*

7. A trainee observes the therapist deliberately shares limited personal information when it is likely to benefit the client rather than meet the therapist's own needs. This is an example of:

A. Self-disclosure

- B. Adjourning stage
- C. Storming stage
- D. Progress review

Answer: A - Self-disclosure. Self-disclosure should be purposeful, brief and client-centered.

Memory aid: *WAIT: Why am I telling this?*

8. A trainee observes a collaborative written or explicit plan linking identified problems to goals, measurable objectives, interventions and review. This is an example of:

A. Treatment plan

- B. Forming stage
- C. Paraphrasing
- D. Group role

Answer: A - Treatment plan. Treatment plans organize care around the client's specific needs and should be revised according to progress.

Memory aid: *PGOIR: Problem, Goal, Objectives, Interventions, Review.*

9. In counseling practice, Carlos encounters a situation involving an organized explanation linking the client's presenting problems with relevant history, triggers, maintaining factors, strengths and a rationale for intervention. Which concept BEST applies?

- A. Support group
- B. Cultural imposition
- C. Group role

D. Case formulation

Answer: D - Case formulation. Formulation goes beyond a diagnosis by explaining why this person is struggling in this way at this time and what may help.

Memory aid: *5P option: Presenting, Predisposing, Precipitating, Perpetuating, Protective.*

10. In counseling practice, Grace encounters a situation involving evaluating whether objectives are being achieved and modifying the treatment plan when progress is absent, circumstances change or goals are met. Which concept BEST applies?

A. Progress review

- B. Adjourning stage
- C. Group confidentiality
- D. Support group

Answer: A - Progress review. Treatment planning is dynamic; review links evidence of progress to continuation, modification or termination.

Memory aid: *Review = measure, discuss, revise.*

11. Which counseling/group/cultural concept is BEST described by: ongoing development of awareness, cultural knowledge, clinical skills and meaningful encounters with diverse clients?

- A. Forming stage
- B. Progress review
- C. Cultural competence**
- D. Self-disclosure

Answer: C - Cultural competence. Cultural competence requires reflective awareness plus knowledge, skills and real encounters; it is not achieved by assuming everyone should conform to the therapist's culture.

Memory aid: AKSE: Awareness, Knowledge, Skills, Encounters.

12. Which counseling/group/cultural concept is BEST described by: judging other cultures primarily by the standards of one's own culture and treating one's own as the norm?

- A. Assessment
- B. Professional interpreter use
- C. Ethnocentrism**
- D. Group confidentiality

Answer: C - Ethnocentrism. Ethnocentrism places one's own cultural frame at the center and may distort assessment or communication.

Memory aid: ETHNO-centred = my culture at the centre.

13. A trainee observes pressuring a client to adopt the therapist's or dominant culture's beliefs, values or practices. This is an example of:

- A. Active listening
- B. Cultural imposition**
- C. Case formulation
- D. Storming stage

Answer: B - Cultural imposition. Cultural imposition occurs when the helper pushes their own values or practices onto the client.

Memory aid: IMPOSITION = impose my culture.

14. In counseling practice, Quentin encounters a situation involving using a qualified interpreter rather than relying on a child or untrained friend when language barriers affect care. Which concept BEST applies?

- A. Professional interpreter use**
- B. Group confidentiality
- C. Assessment
- D. Intervention stage

Answer: A - Professional interpreter use. Qualified interpreters improve accuracy, confidentiality and informed consent.

Memory aid: Interpreter = accuracy + confidentiality.

15. In counseling practice, Nadine encounters a situation involving a group emphasizing mutual support, shared experience, acceptance and coping among people facing similar problems. Which concept BEST applies?

- A. Support group**
- B. Adjourning stage
- C. Cultural competence
- D. Cognitive-behavioral group

Answer: A - Support group. Support groups reduce isolation and offer mutual aid through shared experience.

Memory aid: SUPPORT = Similar people Understanding and Providing Peer help.

SECTION B: MODEL SHORT ESSAYS

1. Differentiate psychoeducational groups from skills-development groups. (4mks)

- Psychoeducational groups emphasize teaching information/understanding about a problem or condition.
- Skills groups emphasize practicing specific behaviors or coping/interpersonal skills with rehearsal and feedback.

Memory aid: Education = know; skills = do.

2. Define a treatment plan and list four common elements. (4mks)

- A collaborative roadmap tailored to the client linking assessment/formulation to care.
- Common elements: identified problem(s), goals, measurable objectives, interventions/strategies, responsible persons/frequency as relevant, review/progress criteria.

Memory aid: PGOIR.

3. Explain SMART goals. (2mks)

- Specific
- Measurable
- Achievable
- Relevant/Realistic
- Time-bound

Memory aid: SMART.

SUPERVISED VOCATIONAL QUALIFICATION DIPLOMA - EXAMINATION

Session: Revision Mock Set 10	Duration: 1hr 30mins
Specialty: Psychotherapy	Coefficient: 04
Eliminatory mark: < 14/20	Training reference: VTI-AEDA examination style

SIMULATION OF REAL WORKING SITUATION PAPER - SOLUTION / MARKING GUIDE PRACTICE SET 10

Revision material based on the supplied VTI-AEDA 2025 examination format and the supplied psychotherapy course recordings. Not an official examination paper.

DETAILED MODEL ANSWERS - SIMULATION OF REAL WORKING SITUATIONS

SECTION I

1. Adolescent substance use - anchor: Substance-use assessment with motivational, supportive and safety-focused intervention

Gisele, age 16, has begun using pills and alcohol several times a week after being bullied and reports skipping school. The adolescent says, "It is the only thing that calms me down," and is unsure about stopping.

Model response / marking points (25 marks):

1. Explain confidentiality and its limits for a minor in the relevant setting.
2. Assess substances, quantity/frequency, intoxication/withdrawal, overdose risk, source, polysubstance use and impairment.
3. Assess suicide/self-harm, violence, exploitation and bullying safety.
4. Explore pros/cons and ambivalence without confrontation or shaming.
5. Link use to triggers and functions such as coping with bullying.
6. Elicit the client's own reasons for change and strengths.
7. Provide age-appropriate psychoeducation/harm reduction within scope.
8. Involve caregivers/other professionals according to consent, safety and legal requirements.
9. Refer for specialist substance/medical evaluation when indicated.
10. Create a concrete follow-up and safety plan.

Suggested mark allocation: Award marks for correct formulation/approach, risk and ethical priorities, logical session structure, appropriate technique application, collaboration/homework/follow-up, and professional communication. Credit clinically sound alternatives when clearly justified. Do not award full marks for naming a theory without showing how it is applied to the facts of the vignette.

Memory aid: ASK: Assess use/risk, Seek motivation, Keep safety/referral central.

2. Psychosis and supportive psychotherapy - anchor: Psychosis - supportive engagement, risk assessment and psychiatric collaboration

Henri, 50, has a history of psychosis and recently began sleeping very little, withdrawing from family and saying that neighbors are sending coded messages through the radio. He is suspicious but willing to talk.

Model response / marking points (25 marks):

1. Use calm, respectful engagement and simple clear communication.
2. Assess hallucinations/delusions, disorganization, sleep, medication adherence, substance use and functional decline.
3. Assess suicide, violence, command hallucinations, inability to care for self and acute medical risk.
4. Do not ridicule or aggressively argue with the belief; validate emotion without confirming the delusion.
5. Orient to shared reality gently and focus on distress/coping.
6. Encourage medication/psychiatric review and coordinate with the treatment team.

7. Mobilize family/supports with appropriate consent and safety considerations.
8. Use simple coping and routine goals.
9. Escalate to emergency care if risk or severe decompensation requires it.
10. Document assessment and plan.

Suggested mark allocation: Award marks for correct formulation/approach, risk and ethical priorities, logical session structure, appropriate technique application, collaboration/homework/follow-up, and professional communication. Credit clinically sound alternatives when clearly justified. Do not award full marks for naming a theory without showing how it is applied to the facts of the vignette.

Memory aid: CALM: Connect, Assess risk, Listen without endorsing delusion, Mobilize care.

SECTION II

1. Eating disorder assessment - anchor: Anorexia nervosa presentation - multidisciplinary assessment plus psychotherapy

Isabelle, 57, has lost substantial weight, restricts food, fears weight gain and exercises excessively. Family members report dizziness and fainting. Isabelle insists there is no problem.

Model response / marking points (25 marks):

1. Recognize low-weight restriction/fear of weight gain as consistent with anorexia nervosa.
2. Assess eating patterns, compensatory behavior, body image, exercise, weight trajectory and motivation.
3. Assess medical instability urgently (fainting, dehydration, cardiac risk) and refer for medical evaluation.
4. Assess suicide/self-harm and comorbid depression/anxiety.
5. Use nonjudgmental engagement; avoid power struggles over food.
6. Use psychoeducation and motivational work.
7. Coordinate nutritional/medical care and family involvement where appropriate.
8. Use evidence-based psychotherapy within competence (e.g., CBT-oriented methods/family-based approaches depending on age/context).
9. Set measurable behavioral and psychological goals.
10. Monitor progress and medical risk closely.

Suggested mark allocation: Award marks for correct formulation/approach, risk and ethical priorities, logical session structure, appropriate technique application, collaboration/homework/follow-up, and professional communication. Credit clinically sound alternatives when clearly justified. Do not award full marks for naming a theory without showing how it is applied to the facts of the vignette.

Memory aid: MIND: Medical risk, Intake/eating pattern, Negative body beliefs, Danger/suicide.

2. Emotion dysregulation and self-harm - anchor: Borderline personality features - DBT-informed treatment with safety planning

Jean, 64, describes rapidly shifting intense relationships, fear of abandonment, impulsive spending, recurrent self-cutting and episodes of overwhelming anger followed by shame.

Model response / marking points (25 marks):

1. Assess current self-harm/suicide risk first and establish a safety/crisis plan.
2. Validate suffering while maintaining clear boundaries.
3. Explain the DBT balance of acceptance and change.
4. Teach mindfulness of current emotion/urge.
5. Introduce distress-tolerance alternatives to self-harm.
6. Teach emotion-regulation skills and trigger tracking.
7. Address interpersonal effectiveness and abandonment-related behaviors.
8. Use behavioral chain analysis around a recent self-harm episode.
9. Set treatment hierarchy and collaborative goals.

10. Coordinate psychiatric/medical care when indicated.

Suggested mark allocation: Award marks for correct formulation/approach, risk and ethical priorities, logical session structure, appropriate technique application, collaboration/homework/follow-up, and professional communication. Credit clinically sound alternatives when clearly justified. Do not award full marks for naming a theory without showing how it is applied to the facts of the vignette.

Memory aid: MDEI: *Mindfulness, Distress tolerance, Emotion regulation, Interpersonal effectiveness.*